

CJC-1295 + Ipamorelin (CJC-1295 + NNC 26-0161)

WHAT IS IT

CJC-1295 + Ipamorelin is a combination of two growth hormone secretagogue peptides that work through different pathways to increase your body's natural growth hormone production. This blend has become the standard entry point for growth hormone peptide therapy because it combines the two cleanest compounds in each of the two major growth hormone signaling categories.

CJC-1295 is a synthetic analog of growth hormone releasing hormone (GHRH), which is a signal your hypothalamus naturally sends to your pituitary gland to tell it to produce and release growth hormone. Think of CJC-1295 as building production capacity because you are telling the factory to make more product.

Ipamorelin is a synthetic ghrelin mimetic, meaning it mimics the hormone ghrelin that your gut produces. Ghrelin also signals your pituitary gland to release growth hormone, but through a completely separate receptor than GHRH. Ghrelin triggers immediate release of stored growth hormone and simultaneously suppresses somatostatin, which is the brake on the growth hormone system. Think of ipamorelin as emptying the warehouse because it triggers release of what has already been made and removes the inhibition at the same time.

The reason these two are blended together rather than used separately is synergy. When both pathways fire simultaneously, the growth hormone response is far greater than adding the individual effects together. Research shows this synergistic combination stimulates significantly more growth hormone release than either compound alone, because GHRH increases the number of pituitary cells releasing growth hormone while ghrelin agonists increase the amount released per cell.

The version of CJC-1295 used in this blend is CJC-1295 without DAC (also called Mod GRF 1-29). DAC stands for Drug Affinity Complex, which is a modification that extends the half-life to about a week by binding to blood proteins. The version without DAC clears quickly within minutes, preserving the natural pulsatile pattern of growth hormone release. This matters because your body is designed to release growth hormone in sharp pulses, not as a flat sustained elevation.

Neither peptide is approved by the FDA for human use. They are available through peptide research suppliers.

How It Works

To understand how this blend works, you need to understand the growth hormone production system and why activating both pathways at the same time produces a result that is greater than the sum of its parts.

The Growth Hormone Production System

Your pituitary gland releases growth hormone in pulses throughout the day and night. The largest pulse happens during deep sleep, specifically between 10:00 p.m. and 2:00 a.m. Secondary pulses occur after intense training and during fasted states when insulin is low and ghrelin is high.

Two signals control this release. The first is GHRH (growth hormone releasing hormone), which comes from your hypothalamus and tells your pituitary to synthesize and release growth hormone. The second is ghrelin, produced in your gut, which triggers immediate release of stored growth hormone. Your body also has a built-in brake called somatostatin. When growth hormone and IGF-1 levels rise, the hypothalamus releases somatostatin to slow production and keep things in balance.

The biggest natural growth hormone pulses happen when both signals fire together, which is exactly why your largest pulse occurs during deep sleep when you are fasted, insulin is low, and ghrelin is high. Both signals are present at the same time.

CJC-1295: The Sustained Signal

CJC-1295 without DAC is a modified version of GHRH that is more stable than the natural hormone but still clears quickly. It binds to GHRH receptors on your pituitary gland and triggers growth hormone synthesis and release. Because it clears within minutes, you get a sharp growth hormone pulse that mimics the natural rhythm your body already uses.

Clinical trials showed that CJC-1295 with DAC (the long-acting version) increased mean plasma growth hormone concentrations 2 to 10 fold above baseline after a single injection, with effects lasting 6 or more days. IGF-1 levels increased 1.5 to 3 fold for 9 to 11 days. The version without DAC used in this blend produces sharper, shorter pulses that align more closely with natural physiology.

Ipamorelin: The Clean Pulse

Ipamorelin is a pentapeptide, meaning it consists of just five amino acids. It binds to ghrelin receptors (GHS-R1a) on the pituitary gland and triggers an immediate burst of growth hormone release. It also suppresses somatostatin, which removes the brake on the system.

Ipamorelin has a half-life of approximately 2 hours. Growth hormone release peaks at about 40 minutes after injection and then returns to baseline. What makes ipamorelin stand apart from older ghrelin mimetics like GHRP-2 and GHRP-6 is its selectivity. Ipamorelin does not significantly raise cortisol, prolactin, or ACTH even at doses more than 200 times higher than the effective dose for growth hormone release. This is a major advantage because cortisol breaks down muscle tissue and promotes fat storage, prolactin can cause low libido and breast tissue changes in men, and ACTH compounds the cortisol problem by triggering more cortisol release from your adrenal glands.

The Synergistic Effect

When CJC-1295 and ipamorelin are injected together, you activate both growth hormone signaling pathways simultaneously. This produces a synergistic effect that is dramatically larger than either compound alone. Research on GHRH and ghrelin receptor agonists confirmed that the submaximal dosages of GHRP plus GHRH stimulated GH release synergistically, indicating they act through independent pathways.

The mechanism behind this synergy has three components. First, GHRH increases the number of pituitary somatotroph cells that release growth hormone. Second, ghrelin agonists like ipamorelin increase the amount of growth hormone released per cell. Third, ipamorelin suppresses somatostatin, which removes the brake on the GHRH signal. So you are simultaneously increasing production capacity, triggering release, and removing the inhibition. The result is a growth hormone pulse that is significantly larger and more sustained than you would get from either pathway alone.

From Growth Hormone to IGF-1

Growth hormone itself does not directly build muscle, burn fat, or repair tissue. When growth hormone is released, it travels to your liver, and your liver converts it into IGF-1 (insulin-like growth factor 1). IGF-1 is the molecule that actually does the work: muscle protein synthesis, fat mobilization, tissue repair, collagen production, and cellular recovery. Think of growth hormone as the messenger and IGF-1 as the worker. You need the whole chain functioning for the system to produce results.

Your body has a built-in paradox that it solves with timing. The best growth hormone release happens when you are fasted because insulin is low and ghrelin is high. But the best IGF-1 conversion in your liver requires insulin to be present. Your body handles this naturally during sleep: you fast overnight, get a massive growth hormone pulse, and that growth hormone circulates for a few hours. Then you wake up, eat, insulin rises, and your liver converts the circulating growth hormone into IGF-1.

Preserving the Feedback Loop

Unlike injecting growth hormone directly, which bypasses the pituitary and eventually suppresses natural production, CJC-1295 and ipamorelin work through the signaling system. They send louder signals, but your body still controls the response. The somatostatin feedback loop remains intact, which means your body maintains its natural regulation. This is a fundamental difference from exogenous growth hormone injection, which overrides the system entirely.

Research confirmed that CJC-1295 preserves the natural pulsatile pattern of growth hormone secretion even during continuous stimulation. The frequency and magnitude of growth hormone secretory pulses remained unchanged, though basal trough levels increased 7.5 fold, which contributed to the overall elevation in growth hormone and IGF-1.

Benefits

The benefits of this blend come from elevated growth hormone and downstream IGF-1 production. Individual human clinical trials exist for each compound separately, but no published RCT has tested the combination specifically in healthy adults. The benefits described here are based on the individual compound data, the established synergy between GHRH and ghrelin pathways, and consistent reports from clinical practice and user communities.

Increased Growth Hormone and IGF-1

The primary effect is higher growth hormone output through amplified natural pulsatile release. CJC-1295 clinical trials showed sustained elevations in both growth hormone and IGF-1, with IGF-1 remaining above baseline for up to 28 days with repeated weekly dosing and evidence of cumulative effect.

Improved Sleep Quality

This is consistently the first benefit people notice, usually within the first 1 to 2 weeks. Growth hormone release naturally spikes during deep sleep. By enhancing this process, the blend often deepens sleep quality. Better sleep creates a positive feedback loop because quality sleep further supports growth hormone production and recovery.

Faster Recovery

Higher growth hormone and IGF-1 support protein synthesis and tissue repair. Most people notice reduced soreness and improved recovery between training sessions within 2 to 4 weeks. This is particularly valuable for adults over 35 whose natural growth hormone production has declined.

Fat Loss

Growth hormone increases lipolysis, which is the breakdown of stored fat for energy. Growth hormone activates an enzyme called hormone-sensitive lipase, which breaks down stored triglycerides into free fatty acids that your body can use as fuel. The fat loss effect typically becomes noticeable around weeks 8 to 12 and is most pronounced when nutrition and training are already dialed in.

Connective Tissue and Skin

Growth hormone stimulates collagen production through IGF-1. Over months 3 to 6, users commonly report improvements in skin quality, joint comfort, and tendon resilience. This is the longer play and one of the most underappreciated benefits, especially for men and women over 40 who are dealing with chronic joint stiffness or lingering tendon issues.

Cognitive Function

Growth hormone receptors exist throughout the brain. Some people report improved mental clarity, focus, and mood stability, though published research on the cognitive effects of secretagogue therapy is less robust than the body composition data.

What to Expect

There is no published clinical trial that tracks the specific timeline of the CJC-1295 + Ipamorelin combination in healthy adults. What follows is based on the individual compound data, my personal experience, my clients' results, and consistent reports from user communities.

Timeline

Weeks 1 to 2: The first thing most people notice is sleep. Deeper sleep, falling asleep faster, and waking up more rested. This happens because the blend amplifies the natural growth hormone pulse that occurs during deep sleep. Some people experience temporary sleep disruptions during the first few weeks as their body adjusts to the larger GH pulses, but this typically stabilizes by week 4 to 6.

Weeks 2 to 4: Recovery improvements become noticeable. Less soreness after training, more energy throughout the day, and the ability to train harder without dragging.

Weeks 8 to 12: Fat loss becomes more apparent. You start leaning out more efficiently, especially if your diet and training are already on point. This is not a magic fat burner. It amplifies what you are already doing.

Months 3 to 6: Connective tissue, skin quality, and joint health improvements become apparent. This is the longer play that requires consistency.

What I Experienced

I ran this blend for about 3 months. The biggest thing I noticed was a clear improvement in the quality of my sleep. The muscle and strength gains were negligible for me, but I was already on TRT and not natural by any stretch of the word, so the equation is different when you are already hormonally optimized. For my clients who took it, the two biggest things they noticed were better sleep and more efficient fat loss when using the stack consistently. But those clients were on it for 16 to 24 weeks and really did not see the fat loss benefits until toward the end of their cycle.

Managing Expectations

These are long-term tools. You are not going to take a shot tonight and wake up jacked tomorrow. Growth hormone peptides do not replace testosterone replacement therapy or anabolic steroids for building muscle. If you are expecting steroid-level muscle growth from this blend, you are going to be disappointed. The value is in recovery, sleep, body composition improvement over months, and connective tissue health.

What the Science Shows

Both peptides have been studied in human clinical trials, though they were tested separately rather than as a blend. No published RCT has tested the combination in healthy adults for body composition or performance outcomes.

CJC-1295 Clinical Data

Two randomized, placebo-controlled, double-blind trials enrolled healthy adults aged 21 to 61. After a single subcutaneous injection, growth hormone increased 2 to 10 fold above baseline and stayed elevated for 6 or more days. IGF-1 increased 1.5 to 3 fold for 9 to 11 days. The estimated half-life was 5.8 to 8.1 days. After multiple weekly doses, IGF-1 remained above baseline for up to 28 days with evidence of cumulative effect. No serious adverse reactions were reported at doses of 30 or 60 mcg/kg. This study used CJC-1295 with DAC, not the no-DAC version used in the blend. The mechanism of pituitary stimulation is the same, but the pharmacokinetics differ.

A separate study confirmed that CJC-1295 preserves the natural pulsatile pattern of growth hormone secretion during continuous stimulation. Pulse frequency and magnitude were unchanged, but basal trough growth hormone levels increased 7.5 fold, and overall mean growth hormone levels rose 46% with IGF-1 levels increasing 45%.

Ipamorelin Clinical Data

A dose-escalation trial in 40 healthy male volunteers established ipamorelin's pharmacokinetic profile. Half-life was approximately 2 hours. Growth hormone release peaked at about 40 minutes after injection with dose-dependent response across all tested doses, followed by exponential decline to baseline.

The foundational selectivity study established ipamorelin as the first selective growth hormone secretagogue. Ipamorelin did not release ACTH or cortisol at any dose tested, even at doses more than 200 times higher than the effective dose for growth hormone release. This is the study that separated ipamorelin from GHRP-2 and GHRP-6 and made it the standard pairing compound.

GHRH + Ghrelin Pathway Synergy

A human study in 18 normal men demonstrated that submaximal doses of a ghrelin receptor agonist plus GHRH stimulated growth hormone release synergistically, confirming the two peptides act through independent pathways. This is the mechanistic basis for combining CJC-1295 and ipamorelin. The synergy has been replicated across multiple studies and is well-established in the endocrinology literature.

Important Limitation

No study has directly measured body composition, strength, recovery, or performance outcomes from the CJC-1295 + ipamorelin combination in healthy, resistance-trained adults. The clinical data confirms that the compounds elevate growth hormone and IGF-1 through their respective pathways and that the pathways are synergistic. The downstream benefits (fat loss, recovery, connective tissue) are inferred from the established effects of elevated growth hormone and IGF-1, supported by clinical practice observations and user reports.

What People Report

The following is aggregated from external platforms including Reddit, peptide forums, and clinic testimonials. This is anecdotal data and does not carry the same weight as published research.

Sleep

Improved sleep quality is the most universally reported benefit. Users across forums consistently describe deeper sleep onset within the first 1 to 2 weeks. Some users report temporary sleep disruptions during the first 3 to 4 weeks, including waking up 2 to 3 times per night, which typically stabilizes around week 4 to 6 as the body adjusts to the larger growth hormone pulses.

Body Composition

Users who run the blend for 16 weeks or longer report gradual improvements in body composition, particularly reduced abdominal fat and improved muscle definition. Most users emphasize that results are subtle and progressive, not dramatic. Users who expect fast visible changes are typically disappointed. Those who stay consistent for 4 to 6 months and have their nutrition and training dialed in report the best outcomes.

Recovery

Reduced soreness after training and the ability to train more frequently are commonly reported within the first month. This is one of the benefits that users on TRT or other hormone protocols notice most, since the elevated growth hormone complements the anabolic environment.

Skin and Connective Tissue

Users who run the blend for 3 months or longer report improvements in skin elasticity, reduced joint stiffness, and better tendon comfort. These reports are less common than sleep and recovery improvements because most users do not run the blend long enough to notice them.

Vivid Dreams

Vivid and sometimes intense dreams are frequently reported, particularly in the first few weeks. This is associated with the enhanced deep sleep phases.

Side Effects Users Report

The most commonly reported side effects are mild: injection site redness, mild water retention, occasional tingling or numbness in the hands, and temporary flushing after injection. Most side effects resolve within the first 2 weeks. Users who push doses too high report more pronounced water retention, carpal tunnel symptoms, and joint swelling.

CJC + Ipamorelin vs Tesamorelin

A common question in the community is whether to choose this blend or tesamorelin. The community consensus aligns with the mechanism: both work through the GHRH pathway, but CJC + ipamorelin hits two pathways (GHRH and ghrelin) while tesamorelin hits only GHRH. For muscle building and general optimization, users favor CJC + ipamorelin for the synergistic effect. For targeted visceral fat reduction, users sometimes prefer tesamorelin because that is what the clinical trials measured.

Dosing Protocol

Studied Doses

The CJC-1295 with DAC clinical trials used 30 to 60 mcg/kg as single subcutaneous injections. The ipamorelin dose-escalation trial tested various doses in healthy male volunteers. No published dose-finding study exists for the subcutaneous injection protocols commonly used in practice for the no-DAC blend.

Practical Protocols

The following protocols are not derived from published dose-finding studies. They represent clinical practice patterns and community consensus.

STANDARD PROTOCOL

CJC-1295 (no DAC): 100 to 300 mcg per day

Ipamorelin: 100 to 300 mcg per day

Timing: Once daily, fasted (at least 2 hours after eating)

Preferred timing: Before bed on an empty stomach

Schedule: Daily

Cycle length: 12 to 16 weeks, then 4 weeks off

BY BODY WEIGHT

Under 150 lbs: 100 to 150 mcg of each peptide

150 to 200 lbs: 200 mcg of each peptide

200 to 250 lbs: 250 to 300 mcg of each peptide

Over 250 lbs: 300 mcg of each peptide

SPLIT DOSING (ADVANCED)

Morning (fasted): 100 to 150 mcg of each

Before bed: 100 to 150 mcg of each

This creates two separate growth hormone pulses per day.

RECONSTITUTION (standard blend vial)

Vial size: 10 mg total (5 mg CJC-1295 no DAC + 5 mg Ipamorelin)

Bac water: 2 mL

Concentration: 250 mcg of each peptide per 10 units on insulin syringe

At 250 mcg of each per day, one vial lasts approximately 20 days.

Why Fasting Matters

When you eat, your blood sugar rises and your body releases insulin. Insulin binds directly to the cells in your pituitary gland that produce growth hormone and suppresses their ability to release it. If you inject these peptides while insulin is present in your blood, you are pressing the gas and the brake at the same time. Take the injection at least 2 hours after eating to ensure insulin has cleared.

Timing Based on Your Goal

Once growth hormone is released, it travels to your liver where it gets converted into IGF-1. But your liver needs insulin present to efficiently convert growth hormone into IGF-1. This creates a timing consideration.

If your primary goal is fat loss, injecting before bed makes the most sense. Growth hormone sits in your system overnight and directly mobilizes fat without needing IGF-1 for that process. You sleep fasted, and the growth hormone works on fat burning all night.

If your primary goal is muscle building, consider injecting in the morning about 30 minutes before you eat. You get the full growth hormone release because you are fasted, then your meal provides the insulin needed to convert it into IGF-1 for muscle protein synthesis. The fasting requirement stays exactly the same. What changes is what you do after you inject.

Receptor Desensitization: The 5 On / 2 Off Myth

The old recommendation was 5 days on, 2 days off to prevent receptor desensitization. This protocol has no scientific basis and sits in a useless middle ground that addresses neither form of actual desensitization.

Acute desensitization occurs within minutes of each dose and resolves in approximately 60 minutes. A study using GHRP-6 in perfused rat pituitary cells found that receptor sensitivity fully recovered after just 60 minutes. Two days off does nothing for this because it resolves on its own within an hour of each injection.

Chronic desensitization develops after approximately 16 weeks of continuous use and requires about 4 weeks off to fully reverse. A human study gave 12 elderly subjects twice-daily hexarelin injections for 16 weeks. Growth hormone response declined

significantly by week 16 but returned to baseline after 4 weeks off. Two days off is far too short to reverse this type of desensitization.

The tesamorelin trials that received FDA approval used daily dosing for 26 weeks. The MK-677 trials used daily dosing for two years. In both cases, growth hormone and IGF-1 stayed elevated the entire time.

The 5 on / 2 off protocol likely started with old-school growth hormone protocols when GH was expensive and people took days off to stretch their supply. Then clinics started charging premium prices for peptides, and taking two days off makes a 30-day supply last 6 weeks instead of four, which makes the business model work better for the clinic. The protocol stuck because it benefited the business model, not because the science supported it.

Daily dosing is the evidence-based approach. If you want to prevent chronic desensitization, run continuously for up to 16 weeks followed by 4 weeks off. If you want to stretch your supply on a tight budget, dosing 5 days per week saves about 30% on product cost. That is a valid budget strategy, but understand it has nothing to do with desensitization.

Stacking Context

CJC-1295 + Ipamorelin + TRT

No interaction concerns. The blend complements testosterone replacement therapy by supporting recovery and body composition through a different pathway. The elevated growth hormone and IGF-1 work alongside testosterone's anabolic effects.

CJC-1295 + Ipamorelin + GLP-1 Agonists (Retatrutide, Semaglutide, Tirzepatide)

No interaction concerns. Completely different mechanisms. Can be run concurrently. The GLP-1 handles appetite regulation and metabolic signaling, and the GH blend handles recovery and growth hormone optimization. The only practical consideration is that GLP-1 agonists can suppress appetite significantly, which may affect the insulin needed for IGF-1 conversion. Make sure you are still eating enough to fuel recovery.

CJC-1295 + Ipamorelin + BPC-157 / TB-500

No interaction concerns but different timing requirements. BPC-157 and TB-500 do not require fasting and can be taken at any time. Keep the GH blend on its own fasting schedule and inject BPC-157 or TB-500 whenever is convenient.

CJC-1295 + Ipamorelin + MOTS-c or SS-31

No interaction concerns. Different mechanisms and targets. MOTS-c and SS-31 target mitochondrial function while the GH blend targets the growth hormone axis. Can be run concurrently.

What Not to Stack With

Do not stack with exogenous growth hormone (HGH) or IGF-1. When you inject growth hormone directly, your IGF-1 levels rise, and your hypothalamus releases somatostatin in response. Somatostatin blocks your pituitary's ability to respond to GHRH signals. Research showed that after a single injection of exogenous growth hormone, the GHRH pathway was substantially inhibited and the ghrelin pathway was also blunted. The secretagogues you are paying for cannot do their job because somatostatin is blocking the signal at the pituitary. Secretagogues and exogenous GH are different tools designed for different goals. Secretagogues work within your body's natural limits and keep your feedback loop intact. Exogenous GH or IGF-1 bypasses the feedback loop entirely.

If you are transitioning from exogenous IGF-1 to this blend, allow 2 to 3 days for the IGF-1 to clear and the negative feedback to begin reversing.

Common Questions

Is CJC-1295 without DAC the same as Mod GRF 1-29? Yes. They are the same compound. CJC-1295 without DAC and Mod GRF 1-29 are two names for the same modified GHRH analog. The original CJC-1295 was designed with DAC for extended duration. The no-DAC version is what is used in the blend because it preserves natural pulsatile release.

Do I need to split the dose or can I take it all at once? Once daily works, but splitting gives better results because you catch two natural growth hormone peaks. If you are going to pick one time, bedtime fasted is the best option because it amplifies the largest natural growth hormone pulse that occurs during deep sleep.

Can I use CJC-1295 by itself without ipamorelin? You can, but you are leaving results on the table. CJC alone activates only the GHRH pathway. Adding ipamorelin activates the ghrelin pathway simultaneously, which is where the synergistic effect comes from. CJC alone will still elevate growth hormone, just not to the same degree as the combination.

Should I choose this blend or tesamorelin? It depends on your goal. Both work through the GHRH pathway, but CJC + ipamorelin hits two pathways while tesamorelin hits only one. For overall optimization including sleep, recovery, and muscle support, the blend is the better choice because of the dual-pathway synergy. Tesamorelin has clinical trial data specifically for visceral fat reduction, so if stubborn belly fat is your main concern and you want the compound with the most targeted data for that outcome, tesamorelin is worth considering. But the mechanism is the same GHRH pathway that CJC already covers.

How long do I need to run this to see results? Expect sleep improvements in weeks 1 to 2. Recovery improvements in weeks 2 to 4. Body composition changes around weeks 8 to 12. Connective tissue benefits at months 3 to 6. Most people who are disappointed with the blend stopped too early.

Can I take it every day or do I need days off? Daily dosing is the evidence-based approach. The 5 on / 2 off protocol has no scientific support. Run continuously for up to 16 weeks, then take 4 weeks off if you are concerned about chronic desensitization.

Will this replace TRT for building muscle? No. Growth hormone peptides do not replace testosterone for building muscle. They are recovery and optimization tools. If you are looking for anabolic muscle growth, testosterone is the primary driver. This blend supports that process but does not replace it.

Side Effects

What the Research Shows

CJC-1295 was well tolerated in clinical trials at doses of 30 to 60 mcg/kg. No serious adverse reactions were reported. Common side effects were transient injection site reactions, mild flushing, and headache.

Ipamorelin showed a clean side effect profile in human trials, with no significant elevations in cortisol, prolactin, or ACTH even at supraphysiological doses. This is the primary safety advantage over older ghrelin mimetics.

What Users Report

Common (usually resolve within 2 weeks):

- Injection site redness or irritation
- Water retention (mild)
- Tingling or numbness in fingers or hands (transient)

- Flushing shortly after injection
- Vivid dreams
- Mild lightheadedness (more common at higher doses)

Less common:

- Headache
- Mild increased hunger
- Fatigue during initial adaptation
- Temporary blood sugar fluctuations

Warning Signs That You Have Gone Too Far

If you notice persistent numbness or tingling in your hands, carpal tunnel symptoms, joint swelling, or facial puffiness from water retention, your dose is too high. Reduce the dose or stop temporarily. More is not better with growth hormone peptides. These symptoms are your body telling you the growth hormone elevation is exceeding what your system can handle cleanly.

Contraindications and Precautions

Do Not Use If You Have:

- Active cancer or tumors. Growth hormone and IGF-1 can accelerate tumor growth. This is a non-negotiable contraindication. If you have any history of cancer, do not use these compounds without clearance from your oncologist.
- Diabetic retinopathy
- Uncontrolled diabetes
- Pregnancy or breastfeeding

Use Caution With:

- Type 2 diabetes or prediabetes (growth hormone can affect insulin sensitivity, monitor blood sugar)
- History of carpal tunnel syndrome
- Cardiovascular disease or high blood pressure
- Kidney or liver disease

Drug Interactions:

- Insulin and oral diabetes medications: growth hormone can alter insulin sensitivity, which may require medication adjustments. Work with your physician.

- Glucocorticoids (prednisone and similar): can blunt the growth hormone response.
- Exogenous growth hormone or IGF-1: do not combine. Somatostatin elevation will block the secretagogue pathway.

Bloodwork to Monitor:

- IGF-1 levels (this is your primary marker for whether the peptides are working and whether levels are in a safe range)
 - Fasting glucose and HbA1c (monitor insulin sensitivity)
 - Standard metabolic panel
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Storage and Handling

CJC-1295 + Ipamorelin is supplied as a lyophilized (freeze-dried) powder that must be reconstituted with bacteriostatic water before use.

RECONSTITUTION

1. Remove caps from both the peptide vial and bacteriostatic water
2. Wipe tops with alcohol swab
3. Draw 2 mL of bacteriostatic water into a syringe
4. Inject slowly against the side of the peptide vial (do not spray directly onto powder)
5. Let the powder dissolve naturally (do not shake)
6. Gently swirl if needed after a few minutes

STORAGE

Before reconstitution: Room temperature or refrigerated

After reconstitution: Refrigerated (36 to 46 degrees F / 2 to 8 degrees C)

Do not freeze after reconstituting.

Use within 4 to 6 weeks of reconstitution.